

Male Genitourinary Physical Exam Checklist

Diagnostic Medicine III 2025

STARTING THE PATIENT ENCOUNTER		
Introduction, Verify Patient		
General Survey		
Vital Signs (obtain or repeat abnormal vitals)		
MALE GENITALIA		
Chaperone	Verbalize "I will have a chaperone present."	
Consent	Verbalize "I need to examine your genitalia and rectum. It may be uncomfortable but it should not be painful. Do you have any questions before we begin?"	
Patient Position	Patient may be standing (preferred) or lying supine with proper draping	
Inspection of pubic region and penis (verbalize)	Pubic hair for distribution, characteristics or infestations (verbalize) IDC	
	Skin for excoriations, lesions, erythema, signs of inflammation (verbalize) LIEE	
	Penis for deformities	
	Foreskin: retract or ask the patient to retract foreskin if uncircumcised, replace or ask patient to replace the foreskin following exam: chancres, carcinomas, smegma (verbalize) COS	
	Glans of penis for (verbalize) ulcers, scars, nodules or signs of inflammation, color, smegma, balanitis, chancre, herpes, warts, cancer	
Palpation of Penis (verbalize)	Urethral Meatus for: (verbalize) position, discharge, erythema	
	(verbalize) Shaft of the penis for any abnormality noting any tenderness and induration	
Inspection of Scrotum (verbalize)	If penile discharge is present, may ask the patient to express the discharge from the shaft of the penis, by applying pressure from its base to the glans to obtain a sample of discharge, observing color and consistency of discharge (verbalize). If necessary the provider may do this themselves once permission obtained.	
	Skin of scrotum for: (verbalize) color, texture, unusual thickening, rashes, epidermoid cysts	CUT TKE
	Scrotum and scrotal contour for: (verbalize) asymmetry, swelling, lesions, veins, hernias, hydrocele, cryptorchidism, lumps and masses	
Palpation of Scrotum and Testes (verbalize)	Elevate scrotum to visualize posterior aspect. May ask patient to lift scrotum.	
	Testes and Epididymis (located superior posterior surface of testes) for: (verbalize) size, shape, consistency, tenderness, swelling, nodules and masses	
	(verbalize) Scrotum for lesions or edema	
Transillumination (verbalize)	(verbalize) Spermatic cord, including vas deferens for nodules and masses, swelling,	
	If mass or enlargement in scrotum is noted, use light source to transilluminate scrotal mass. (Transillumination will occur if the mass is fluid filled)	
INGUINAL REGION (bilaterally)		
Inspection (verbalize)	Adenopathy, erythema ↳ Inguinal region is "a-den" and "errio"	
Palpation (verbalize)	Lymph nodes for: size, consistency, tenderness (verbalize)	
HERNIAS (bilaterally)		
Inspection (verbalize)	Patient is standing, provider is sitting in front of patient	
	Inspect the inguinal regions and genitalia	
	Ask patient to bear down observing for: asymmetry or bulging	
Palpation (verbalize)	To palpate for an inguinal hernia: Insert your examining finger (index finger) into the lower part of the scrotum close to the inferior margin and carry it upward along the vas deferens	
	Ask patient to bear down observing for: Hernia or mass	

RECTUM		
Inspection (verbalize)	Patient lying in lateral decubitus position with knees bent or standing leaning forward over exam table	
	Sacroccygeal, gluteal cleft and perianal area for: (verbalize) pilonidal dimpling or tuft of hair, erythema, excoriations, lumps, ulcers, inflammation, rashes	
	Anus for: (verbalize) changes in skin, lesions, fissures, hemorrhoids, fistulae, rectal prolapse	
Palpation (verbalize)	Apply a small amount of lubrication to gloved index finger. Apply gentle pressure with index finger at the sphincter, once the sphincter has relaxed, slowly insert index finger into rectum. Note <u>smoothness</u> and <u>evenness</u> of muscular <u>ring</u>	
	Slowly rotate finger clockwise direction and then counterclockwise evaluating rectal walls for (verbalize) masses and nodules, tenderness, irregularities, induration IM IT	
	Inter-rectal palpation of the prostate: rotate hand further counterclockwise so that your finger can examine the posterior surface of the prostate gland. Sweep finger carefully over the prostate gland identifying lateral lobes and median sulcus between them. Then if possible, extend your finger above the prostate to the region of the seminal vesicles and the peritoneal cavity and sweep the anterior wall. (verbalize) Note symmetry of lobes, size, shape, contour, consistency, irregularities, tenderness, masses and nodules IM IT SSCC	
Assess sphincter tone (verbalize)	Note sphincter tone of the anus	
	To check for external sphincter tone, ask pt to bear down & squeeze rectal muscles	
	(verbalize) Note for any tenderness, induration, irregularities and nodules IM IT	
Completion of Exam (verbalize)	Note smoothness and evenness of muscular ring like beginning, but now while bearing down	
	Gently withdraw your finger and note color, presence of blood, consistency of any fecal matter on your glove, and test it for occult blood. Wipe the anus or give the patient tissues.	
	Remove and dispose gloves	
	Offer to help the patient cover up and sit up	
	Wash hands with soap and water	